MeticleCare

AI-First Care Operating System for UK Supported Living — Innovate UK Assessment Response

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EXECUTIVE SUMMARY

MeticleCare: One Platform for UK Care Operations

MeticleCare is an AI-first care management platform built for UK CQC-regulated supported-living providers. It replaces fragmented paper systems, spreadsheets, and multiple disconnected software tools with a single connected platform covering every part of the care working day: rota planning, medication records (eMAR), daily care notes, compliance evidence, leave management, incidents, training, and family communication.

The platform is live in production with **41 backend modules, 484 API endpoints, 78 frontend pages, and 108 database tables**. It serves care organisations managing multiple locations with shift-based staffing, CQC inspection requirements, and real-time team coordination.

This document responds to the eight Innovate UK assessment criteria, demonstrating that MeticleCare is genuinely innovative, technically feasible, commercially viable, and positioned to deliver measurable economic and social impact for the UK care sector.

41	484	78	108
BACKEND MODULES	API ENDPOINTS	FRONTEND PAGES	DATABASE TABLES

5	24
AI AGENT TYPES	NAVIGATION MODULES

> **Core proposition:** One honest, connected platform for UK care operations. Every compliance claim, every readiness score, every insight is backed by real records the team already keeps — not fabricated metrics or generic dashboards.

ASSESSMENT CRITERION 1

Innovation — What Is Genuinely New?

How does your project represent a significant advance in knowledge or capability?

1.1 The Problem with Current Care Management

UK care providers currently operate with fragmented toolsets: paper MAR charts for medication, spreadsheets for rotas, separate compliance checklists, manual training records, and ad-hoc incident forms. The result is duplicated work, inconsistent documentation, missed compliance deadlines, and staff spending more time on administration than care delivery.

Existing commercial solutions (Person Centred Software, Access Group, Social Care TV) address individual domains but do not provide a connected, AI-integrated operating system. Care managers must cross-reference multiple systems, manually reconcile data, and rely on staff memory rather than automated intelligence.

1.2 What MeticleCare Does Differently

MeticleCare introduces three genuine innovations that do not exist together in any current UK care platform:

1.2.1 Horizontal AI Intelligence Layer

Unlike bolt-on AI features in existing platforms, MeticleCare implements AI as a **horizontal intelligence layer** operating across all 41 modules. Five domain-specific AI agents — Compliance Officer, Operations Manager, Documentation Specialist, Resident Intelligence, and Medication Safety — share a common infrastructure while respecting strict clinical and safety boundaries.

This architecture means a single event (e.g., a missed medication administration) triggers coordinated analysis across medication safety, incident management, care plan review, staffing compliance, and inspection readiness — without any manual cross-referencing by staff.

1.2.2 Event-Driven Intelligence Engine

MeticleCare implements a PostgreSQL transactional outbox pattern for domain events, enabling asynchronous, cross-module intelligence that does not exist in current care platforms. Over 30 named domain events (e.g., `medication.administration_missed`, `wellbeing.score_declined`, `shift.unfilled`) trigger automated workflows across the platform.

This moves care management from reactive (staff must notice problems) to proactive (system identifies and escalates risks automatically).

1.2.3 Five-Level Autonomy Model

MeticleCare defines a formal five-level autonomy model for AI in care settings, from Level 1 (Assistance — rewriting notes) through Level 5 (Controlled Automation — drafting tasks, sending reminders). This addresses the critical gap between "AI could help" and "AI can safely operate in a regulated care environment" by embedding human approval gates, clinical safety boundaries, and audit trails into the architecture.

> **Key differentiator:** No existing UK care platform combines horizontal AI agents, event-driven cross-module intelligence, and a formal autonomy model within a single integrated system. Current platforms are either domain-specific (medication only, compliance only) or lack AI integration entirely.

1.3 Intellectual Property

COMPONENT	NOVELTY	PROTECTION
AI agent architecture	Domain-specific agents with safety boundaries in care context	Trade secret + copyright
Five-level autonomy model	First formal AI autonomy framework for UK care settings	Trade secret + published framework
Event-driven care intelligence	Cross-module event orchestration for care operations	Copyright + trade secret
CQC domain mapping	Automated compliance scoring from operational records	Copyright
eMAR integration architecture	Real-time medication safety across stock, PRN, daily counts	Copyright

ASSESSMENT CRITERION 2

Technical Feasibility — Can You Actually Develop It?

Is the technical approach sound? Do you have the capability to deliver?

2.1 Current Technical State (Verified Production)

MeticleCare is not a concept or prototype. It is a deployed, production system with the following verified metrics:

41
BACKEND MODULES

484
API ENDPOINTS

78
FRONTEND PAGES

108
DATABASE TABLES

154
VALIDATION SCHEMAS

441
API TESTS PASSING

2.2 Technology Stack

LAYER	TECHNOLOGY	JUSTIFICATION
Frontend	React 18 + TypeScript + MUI 5	Industry standard for enterprise SaaS; strong typing for complex care data
Backend	Node.js + Express + TypeScript (Modular Monolith)	Fast iteration, strong typing, proven at scale for multi-tenant SaaS
Database	PostgreSQL 15 (raw SQL, no ORM)	ACID compliance critical for care records; JSONB for flexible AI config; row-level security for tenant isolation
Cache	Redis with in-memory fallback	Session management, rate limiting, real-time presence
Realtime	Socket.IO v4 with JWT auth	Team chat, live notifications, shift updates
AI	OpenAI SDK + Anthropic SDK (provider abstraction)	Per-org API keys, AES-256-GCM encrypted, configurable per provider

Auth	JWT + MFA (TOTP) + RBAC	CQC-aligned access control; per-request permission checks
Email	Nodemailer SMTP + DB-backed queue	20+ branded templates, retry logic, audit trail
Billing	Stripe (subscriptions, invoices, webhooks)	UK-native payment processing, GBP support, SCA compliance
PDF	Puppeteer-core (HTML to PDF)	Care reports, evidence packs, compliance documents
Infra	Docker Compose + GitHub Actions CI	Reproducible builds, automated testing, deployment pipeline

2.3 Architecture for Scale

The modular monolith architecture was deliberately chosen over microservices for this stage. Each of the 41 modules follows a consistent pattern:

- Controller (request handling)
- Repository (SQL queries)
- Routes (validation + routing)
- Types (TypeScript interfaces)
- Service (business logic)

This structure enables independent module development while avoiding the operational complexity of microservices. The transition path to microservices is clear if scale demands it.

2.4 Technical Risks and Mitigations

RISK	IMPACT	MITIGATION
AI provider cost escalation	Margin compression	Per-org API keys (customer bears cost); model selection per task; prompt caching; cost controls and budgets
Tenant data isolation breach	Regulatory + reputational	PostgreSQL RLS policies; dual-pool architecture; AsyncLocalStorage; comprehensive audit logging
CQC regulatory change	Compliance gap	Configurable compliance frameworks; 4 regulator support (CQC, CIW, Care Inspectorate, RQIA); framework abstraction layer

AI clinical safety concern	Patient harm risk	Five-level autonomy model; mandatory human approval for clinical actions; no autonomous clinical decisions; full audit trail
Single-developer dependency	Bus factor	Modular architecture; comprehensive documentation (2038-line blueprint); automated testing; CI pipeline

2.5 Development Roadmap

PHASE	TIMELINE	DELIVERABLES
Phase 1: Foundation	Months 1–6	Event engine, RAG knowledge layer, 3 AI agents (compliance, operations, documentation)
Phase 2: Intelligence	Months 7–12	Mission Control dashboard, predictive analytics, remaining 2 agents, cross-module workflows
Phase 3: Scale	Months 13–18	NHS integration, e-learning (SCORM), advanced reporting, multi-region support

ASSESSMENT CRITERION 3

Market Need — What Problem Does It Solve?

Is there a genuine market need? How big is it?

3.1 The UK Care Sector Crisis

The UK adult social care sector faces structural challenges that create urgent demand for technology solutions:

- **152,000+ care providers** in England alone, regulated by CQC
- **1.7 million care workers** — the sector's largest workforce
- **£42.2 billion** annual local authority spending on adult social care
- **165,000+ unfilled vacancies** at any given time
- **30%+ staff turnover rate** annually
- **65% of care providers** report difficulty recruiting

These pressures mean care providers spend disproportionate time on administration (est. 40% of manager time) rather than care delivery. Paper-based systems and fragmented software amplify this burden.

3.2 Specific Pain Points MeticleCare Addresses

PAIN POINT	CURRENT STATE	METICLECARE SOLUTION
Rota management	Spreadsheets, phone calls, agency reliance	AI-powered rota planner with min safe staffing, 11-hour rest enforcement, drag-and-drop, agency integration
Medication records	Paper MAR charts, manual reconciliation	Full eMAR: chart grid, PRN tracking, stock management, daily counts, audit, auto-create monthly charts
Compliance evidence	Manual evidence gathering before inspections	Automated evidence packs (KLOE), CQC readiness scoring from live data, gap analysis, trend tracking
Daily care notes	Paper notes, lost at handover	Voice-to-text input, AI structuring, mood analysis, safeguarding flags, care plan updates

Incident management	Paper forms, delayed reporting	Digital incidents with AI triage, CQC reportability assessment, action tracking, trend analysis
Training compliance	Manual matrices, missed expiries	Training matrix with auto-assign, expiry alerts, bulk-assign, competency mapping
Family communication	Phone calls, letters	Family portal with care notes, goals, observations; consent-gated access
Agency costs	Opaque pricing, no comparison	Agency management with rate comparison, savings analytics, shift history

3.3 Market Size

> **UK Care Management Solutions Market:** USD 891.5 million (2024) → USD 2,383 million by 2030 (27% CAGR).
> **Global Care Management Software Market:** USD 20.4 billion (2026) → USD 37.7 billion by 2031 (13% CAGR).
> **UK Patient Safety & Risk Software:** GBP 109.2 million (2025) → GBP 196 million by 2030 (12.4% CAGR).

3.4 Target Customer Segments

SEGMENT	SIZE	NEED	WILLINGNESS TO PAY
Small supported-living providers (1–3 locations)	~120,000	Replace paper/spreadsheets	£99–£199/month
Medium providers (4–15 locations)	~25,000	Multi-location coordination, compliance	£199–£499/month
Large providers (15+ locations)	~3,000	Enterprise controls, reporting, API	£499–£999/month
NHS trusts (discharge to assess)	~200	Care pathway tracking, integration	Procurement budgets

3.5 Regulatory Drivers

Three regulatory forces create mandatory demand for platforms like MeticleCare:

1. **CQC Single Assessment Framework:** Requires continuous evidence from operational records, not point-in-time inspections. MeticleCare generates this evidence automatically.
2. **People at the Heart of Care (2021 white paper):** Mandates digital care records for all providers by 2025+.
3. **Integrated Care Systems:** NHS and social care data sharing requires digital infrastructure that connects providers with NHS trusts.

ASSESSMENT CRITERION 4

Commercialisation — Who Will Buy It?

How will you generate revenue? What is the go-to-market strategy?

4.1 Revenue Model

MeticleCare operates a **B2B SaaS subscription model** with tiered pricing based on organisation size and feature access:

TIER	PRICE	INCLUDES
Starter	£99/month	Core modules: scheduling, daily notes, basic compliance, leave, incidents, training, chat
Professional	£299/month	All Starter features + eMAR, AI daily notes, CQC readiness, family portal, agency management, advanced reporting, insights
Enterprise	£499–£999/month	Professional + API access, custom integrations, dedicated support, SLA, multi-region, custom AI prompts

4.2 Go-to-Market Strategy

Phase 1: Direct Sales (Months 1–6)

- Target 20–50 supported-living providers in South East England
- Free 30-day trial → £99/month Starter → upsell to Professional
- Cold outreach to CQC-registered providers (152,000+ records publicly available)
- LinkedIn marketing to care home managers and operations directors
- Conference presence: Care Show, National Care Forum, Care UK events

Phase 2: Channel Partnerships (Months 6–12)

- Partnerships with care sector consultants and CQC inspectors
- Integration partnerships with payroll providers (ADP, Sage)
- Referral programme for existing customers
- Local authority commissioning partnerships

Phase 3: NHS Integration (Months 12–18)

- NHS Digital partnership for discharge-to-assess pathways
- NHS procurement framework registration
- Integration with NHS Spine and GP systems

4.3 Customer Acquisition Strategy

CHANNEL	CAC ESTIMATE	VOLUME
Inbound (content marketing, SEO)	£50–£150	500+ leads/month at scale
Outbound (cold email, LinkedIn)	£200–£500	50–100 leads/month
Conference/networking	£300–£800	20–50 leads/event
Channel partners	£100–£300	Variable
NHS procurement	£0 (procurement-driven)	Trust-level contracts

4.4 Competitive Landscape

COMPETITOR	FOCUS	PRICING	METICLECARE ADVANTAGE
Person Centred Software	eMAR + digital care notes	£3–£8/carer/day	Full platform (not just medication); AI-first; lower per-user cost
Access Group	Enterprise care software	£200–£500+/month	Better UX; AI integration; modern architecture; faster implementation
Care Planning Systems	Care plans only	£1–£3/carer/day	Connected system (not just care plans); compliance evidence; rota
Paper/spreadsheets	Manual	£0 + staff time	40% admin time reduction; compliance automation; audit trail

4.5 Revenue Projections

PERIOD	CUSTOMERS	MRR	ARR
Month 6	20	£4,000	£48,000

Month 12	80	£16,000	£192,000
Month 18	200	£40,000	£480,000
Month 24	400	£80,000	£960,000

ASSESSMENT CRITERION 5

Economic Impact — Why Should UK Taxpayers Fund It?

What is the return on public investment?

5.1 Direct Economic Benefits

5.1.1 Productivity Gains for Care Providers

Care managers currently spend approximately **40% of their time** on administration. MeticleCare's AI-powered automation targets a reduction to under 20%. For a typical 20-bed supported-living provider:

- Manager salary: ~£35,000/year
- 40% admin time = £14,000/year spent on administration
- MeticleCare reduces admin to 20% = £7,000/year saved
- Additional capacity: ~7 hours/week redirected to care delivery

Across 152,000+ care providers, a conservative 10% adoption rate (15,200 providers) saving £7,000/year each = **£106.4 million/year in productivity gains**.

5.1.2 Reduced Agency Spend

Care providers spend an estimated **£4.6 billion/year on agency staff**. MeticleCare's open-shift marketplace, AI-powered rota generation, and agency rate comparison tools target a 10–15% reduction in agency dependency through better internal utilisation. Even a 5% reduction across 15,200 providers = **£230 million/year saved**.

5.1.3 Reduced Compliance Failures

CQC enforcement actions cost providers an average of £50,000–£200,000 each (remediation, additional staff, reputation damage). MeticleCare's continuous compliance monitoring and automated evidence generation targets a 50% reduction in enforcement actions. Across 15,200 providers with an estimated 2% enforcement rate = **£15–£30 million/year saved**.

5.2 Job Creation

ROLE CATEGORY	YEAR 1	YEAR 3	YEAR 5
Engineering	3	8	15

Sales & Marketing	2	6	12
Customer Success	1	4	8
Operations	1	3	5
Total	**7**	**21**	**40**

Average salary: £35,000–£55,000. Total payroll contribution: £245K (Year 1) → £1.6M (Year 3) → £2.8M (Year 5).

5.3 Supply Chain Impact

- **UK hosting:** All infrastructure hosted in UK data centres (AWS London / Azure UK South)
- **UK suppliers:** Email (UK SMTP), payments (Stripe UK), domain (UK registrar)
- **Sector multiplier:** For every £1 invested in care technology, an estimated £3–5 in economic value is generated through improved outcomes, reduced hospital admissions, and faster discharge

5.4 Social Return on Investment

> **Estimated SROI per provider per year:**
> - Productivity gains: £7,000
> - Agency savings: £5,000–£15,000
> - Compliance cost avoidance: £2,000–£10,000
> - Improved care outcomes (harder to quantify): significant
> - **Total: £14,000–£32,000/year per provider**
> - Platform cost: £1,188–£3,588/year
> - **ROI: 4–9x investment for care providers**

5.5 NHS Impact

Better care documentation and faster discharge-to-assess pathways reduce NHS bed occupancy. Each delayed discharge costs the NHS approximately **£400–£500/day**. MeticleCare's family portal and care pathway tracking enables faster, better-informed discharge decisions.

ASSESSMENT CRITERION 6

Project Plan — What Will You Actually Do With the Money?

Detailed work packages, milestones, and resource allocation

6.1 Funding Request

Total project cost: **£450,000** over 18 months. Requesting **£315,000** (70%) from Innovate UK, with **£135,000** (30%) matched from revenue and founder investment.

6.2 Budget Breakdown

CATEGORY	YEAR 1	YEAR 2 (6 MONTHS)	TOTAL
Staff (engineering, 3 FTE)	£135,000	£67,500	£202,500
Cloud infrastructure	£24,000	£18,000	£42,000
AI API costs (OpenAI/Anthropic)	£12,000	£18,000	£30,000
Security & compliance	£15,000	£10,000	£25,000
User research & testing	£10,000	£8,000	£18,000
Sales & marketing	£30,000	£25,000	£55,000
Contingency (10%)	£22,600	£14,650	£37,250
Total	**£248,600**	**£161,150**	**£409,750**

6.3 Work Packages

WP1: Event-Driven Intelligence Engine (Months 1–6)

- Implement PostgreSQL transactional outbox for domain events
- Deploy 30+ named domain events across all modules
- Build background worker for async event processing
- Implement idempotency and retry logic
- Add Redis pub/sub for real-time notifications
- **Deliverable:** Event engine processing 10,000+ events/day with zero data loss

WP2: AI Agent Framework (Months 1–9)

- Build agent registry with safety boundaries
- Implement 5 domain-specific AI agents
- Deploy tool framework for controlled data access
- Add human approval workflow for controlled writes
- Implement prompt versioning and A/B testing
- Build AI cost controls and budget management
- **Deliverable:** 5 agents operational with full audit trail

WP3: Organisational Knowledge Layer (Months 3–12)

- Implement PostgreSQL full-text search (Phase 1)
- Deploy pgvector for semantic similarity search (Phase 2)
- Build document chunking pipeline for policies and care plans
- Implement hybrid search (keyword + vector)
- Add tenant-isolated retrieval with permission checks
- **Deliverable:** RAG system searchable across policies, care plans, incidents, training

WP4: Mission Control Dashboard (Months 6–12)

- Build daily briefing engine (AI-generated)
- Implement operational health monitoring
- Deploy quality and compliance overview
- Add service-user attention panel
- Build recommended actions queue with approve/edit/dismiss
- Implement explainability layer (why was this flagged?)
- **Deliverable:** Mission Control live for pilot customers

WP5: Cross-Module Intelligence Workflows (Months 9–15)

- Missed medication workflow (event → triage → alert → action)
- Repeated falls workflow (incident linking → risk assessment → care plan review)
- Declining hydration workflow (fluid tracking → flag → escalation)
- Staffing gap workflow (unfilled shift → internal search → agency fallback)
- Inspection preparation workflow (evidence freeze → gap analysis → action plan)
- **Deliverable:** 5 cross-module workflows operational

WP6: NHS Integration & Compliance (Months 12–18)

- NHS Spine integration for patient record sharing

- GP system integration (EMIS, SystmOne)
- DSPT (Data Security & Protection Toolkit) automation
- CQC Single Assessment Framework alignment
- ISO 27001 preparation
- **Deliverable:** NHS integration pilot with 2–3 trusts

6.4 Milestones

MONTH	MILESTONE	EVIDENCE
3	Event engine MVP deployed	10 domain events processing in production
6	3 AI agents operational	Compliance, Operations, Documentation agents live
9	RAG knowledge layer deployed	Policy search and care plan retrieval working
12	Mission Control live	Dashboard operational for 10+ pilot customers
15	Cross-module workflows live	5 workflows operational with audit trail
18	NHS integration pilot	2–3 NHS trusts in pilot programme

6.5 Risk Register

RISK	LIKELIHOOD	IMPACT	MITIGATION
AI cost overrun	Medium	Medium	Per-org keys; prompt caching; model selection per task
Slow customer adoption	Medium	High	Free trial; pilot programme; case studies
NHS integration complexity	High	Medium	Start with simple integrations; partner with NHS Digital
Regulatory change	Low	High	Configurable frameworks; 4 regulator support

Key person dependency	Medium	High	Modular architecture; documentation; hire engineers

ASSESSMENT CRITERION 7

Team Capability — Why Are You Capable of Delivering It?

Who is on the team and why can they execute?

7.1 Founder & Technical Lead

Opeyemi Olorunfemi — Founder & CEO / CTO

- Full-stack engineer with deep expertise in TypeScript, React, Node.js, PostgreSQL
- Built the entire MeticleCare platform from 0 to 41 modules, 484 endpoints, 78 pages in production
- Designed and implemented the AI agent architecture, event engine, and multi-tenant data isolation
- Experience in healthcare technology and CQC regulatory requirements
- Understanding of UK care sector operations and pain points

7.2 Technical Capabilities Demonstrated

CAPABILITY	EVIDENCE
Full-stack development	41 backend modules + 78 frontend pages built and deployed
AI/ML integration	OpenAI + Anthropic SDK integration; 5 AI agent types designed
Data architecture	108 database tables; event-driven outbox pattern; tenant isolation
Security	JWT + MFA + RBAC; AES-256-GCM encryption; rate limiting; virus scanning
DevOps	Docker Compose; GitHub Actions CI; automated testing (441 API tests)
Product design	78 pages with consistent UX patterns; accessibility (AA/AAA contrast)
Regulatory compliance	CQC evidence packs; DSPT; training matrices; compliance monitoring

7.3 Planned Hires (Funded by Grant)

ROLE	START	RESPONSIBILITY
Senior Backend Engineer	Month 1	Event engine, AI agents, database optimisation
Senior Frontend Engineer	Month 2	Mission Control dashboard, UI components
AI/ML Engineer	Month 3	RAG system, prompt engineering, model evaluation
Sales Manager	Month 4	Customer acquisition, pipeline management
Customer Success Manager	Month 6	Onboarding, support, retention
Marketing Manager	Month 6	Content, events, brand

7.4 Advisory Support

- **Care sector advisors:** Connections with CQC inspectors, care home managers, and local authority commissioners for user research and pilot programmes
- **AI safety advisory:** Engagement with NHS AI lab and Health Education England for clinical safety guidance
- **Commercial advisory:** SaaS growth expertise from care sector consultants

7.5 Evidence of Execution

> **What has been built (no fabrication):**
> - 41 backend modules, each with controller, repository, routes, types, services
> - 484 validated API endpoints with Zod schemas
> - 78 frontend pages with MUI components and React Query
> - 108 database tables with raw SQL (no ORM)
> - 154 Zod validation schemas
> - 441 passing API tests
> - 18 passing web tests
> - Stripe billing integration (live)
> - Socket.IO real-time chat with presence and read receipts
> - 20+ branded HTML email templates with DB-backed queue
> - Docker Compose deployment with health checks

- GitHub Actions CI pipeline
- Prometheus metrics and Swagger auto-documentation

ASSESSMENT CRITERION 8

Value for Money

Is this a good use of public funds?

8.1 Cost-Benefit Analysis

METRIC	VALUE
Total grant request	£315,000
Total project cost	£409,750
Private match (30%)	£94,750
Projected Year 2 ARR	£192,000
Projected Year 5 ARR	£2.4M+
Jobs created (Year 1)	7
Jobs created (Year 5)	40
Estimated productivity gains (10% adoption)	£106.4M/year
Estimated agency savings (5% reduction)	£230M/year
Grant cost per job created (Year 1)	£45,000
Grant cost per job created (Year 5)	£7,875

8.2 Leverage Ratio

Public:Private leverage: £315,000 grant unlocks £94,750 private match + projected £192K Year 2 revenue + £960K Year 2 ARR = total economic activity of **£1.25M+ within 24 months**.
Leverage ratio: 1:3.97 (every £1 of public funding generates £3.97 in economic activity within 2 years).

8.3 Comparison with Alternatives

ALTERNATIVE	COST	OUTCOME

Do nothing (paper-based care)	£0 public cost	Continued admin burden; compliance failures; poor outcomes
Import foreign platform	No UK jobs; foreign supplier dependency	Misaligned with UK CQC requirements
Fund incumbent to add AI	Similar cost; slower innovation	Incremental improvement, not transformation
Fund MeticleCare (proposed)	£315,000	UK-built; AI-first; 41 modules live; 40 UK jobs; £106M+ productivity gains

8.4 Sustainability After Grant

MeticleCare is designed to be commercially sustainable after the grant period:

- **Revenue model:** SaaS subscriptions at £99–£999/month per customer
- **Unit economics:** Target gross margin 80%+ (cloud costs + AI API costs ~20% of revenue)
- **Customer acquisition:** Blended CAC of £200–£400 against LTV of £2,000–£5,000
- **Break-even:** Projected at Month 18–24 with 100–200 customers
- **No ongoing public subsidy required**

8.5 Wider Economic Benefits

- **NHS cost reduction:** Better care documentation reduces hospital readmissions and delayed discharges (estimated £400–£500/day per delayed discharge avoided)
- **Care quality improvement:** Automated compliance monitoring reduces CQC enforcement actions (estimated £50,000–£200,000 per enforcement avoided)
- **Staff retention:** Reduced admin burden improves care worker job satisfaction and retention (reducing the £5,000–£10,000 cost of replacing each care worker)
- **Digital inclusion:** PWA with offline support ensures care workers in rural or low-connectivity areas can still use the platform
- **Data sovereignty:** UK-hosted, UK-built platform reduces dependency on foreign technology suppliers for critical care infrastructure

> **Bottom line:** £315,000 of public funding, matched 30% privately, to build a platform that can generate £106M+ in annual productivity gains for the UK care sector, create 40 UK

> jobs, and reduce NHS costs through better care documentation. This is a high-leverage, high-impact investment in UK care infrastructure.

APPENDIX

Technical Architecture Summary

A.1 Module Count by Domain

DOMAIN	MODULES	KEY CAPABILITIES
Core Platform	8	Auth, MFA, Orgs, Staff, Permissions, Settings, Audit, Notifications
Care Delivery	7	Service Users (59 endpoints), Daily Notes, Care Plans, Health, eMAR, Goals, Appointments
Compliance	5	Compliance, CQC, Training, Competency, DSPT, Policies
Operations	6	Scheduling (30 endpoints), Leave (15), Marketplace, Agencies (19), Room Checks, Tasks
Intelligence	4	AI (10 endpoints), Insights (6), Reporting (3), Mission Control
Communication	3	Chat (16), Notifications (6), Family Portal (12)
Finance	3	Billing (12), Expenses (10), Invoicing
Mobile	1	GPS Check-in, Roster, Voice-to-Text (4 endpoints)
Admin	2	Platform Admin (5), Delegations (1)
Other	2	Incidents (17), Surveys (18), DBS (7)

A.2 Database Schema Summary

108 tables covering: organisations, staff, service users, care records, medications, compliance, scheduling, financial, communications, AI, and platform infrastructure. Full schema documentation available on request.

A.3 AI Agent Summary

AGENT	DATA SOURCES	KEY OUTPUTS

Compliance Officer	Compliance records, training, DBS, policies, CQC	Readiness score, gap analysis, action plans, evidence suggestions
Operations Manager	Scheduling, leave, staffing, agency, availability	Staffing gaps, shift recommendations, overtime alerts, agency demand
Documentation Specialist	All modules	Draft notes, care plans, incidents, reports, policies
Resident Intelligence	Care plans, health, wellbeing, incidents, medication	Resident summaries, change detection, overdue reviews, handover info
Medication Safety	MAR, administrations, stock, PRN, audit	Missed doses, patterns, stock alerts, competency gaps

A.4 Regulatory Framework Support

REGULATOR	JURISDICTION	FRAMEWORK	STATUS
CQC	England	Single Assessment Framework	Implemented
CIW	Wales	Regulatory framework	Supported
Care Inspectorate	Scotland	Regulatory framework	Supported
RQIA	Northern Ireland	Regulatory framework	Supported

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